

Research-Driven Model for Adapting Mental Health Interventions for Neurodivergent Youth
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Step	Adaptation Phase	Key Actions and Embedded Community Engagement
1	Co-Define the Problem and Priorities	Partner with community members and organizations before adaptation begins. Use advisory boards and listening sessions to identify unmet needs and intervention gaps. Ensure community members have shared influence over defining priorities rather than serving only as reviewers. Consult experts in the research field as well to assist in problem determination.
2	Multi-Informant Needs Assessment	Conduct qualitative and quantitative assessments to identify cognitive, behavioral, and systemic mismatches between the original intervention and neurodivergent profiles. Involve community members in shaping interview guides, reviewing accessibility of materials, and interpreting findings to prevent deficit-based misinterpretations.
3	Ground Adaptations in Theory and Living Experience	Anchor adaptations in the intervention's theoretical framework, the social model of disability, and an implementation science framework such as EPIS. Present proposed modifications to community members for feedback to ensure theoretical integrity aligns with real-world relevance.
4	Systematic Adaptation Design and Documentation	Modify language, pacing, structure, visual supports, caregiver integration, and generalization scaffolds based on community needs. Engage community partners in reviewing draft materials and testing usability. Document each adaptation with its rationale, level of change, and anticipated impact on fidelity and feasibility. Utilize a framework to do so, such as MADI. Maintain fidelity as much as possible.
5	Workforce Preparation and Collaborative Training	Provide clinicians with structured training on FASD or other neurodevelopmental characteristics, adaptation rationale, and fidelity–flexibility balance. Incorporate living-experience perspectives when possible.

		Establish ongoing consultation mechanisms informed by community member feedback.
6	Feasibility Trial with Continuous Feedback Loops	Trial the adapted intervention in a real-world setting. Evaluate recruitment, retention, engagement, fidelity, acceptability, and preliminary clinical signals. Conduct structured post-intervention interviews with all members involved in the trial.
7	Iterative Refinement and Sustainment Planning	Use mixed-method findings to refine materials, delivery structures, caregiver supports, and training models. Collaboratively plan for larger-scale testing or integration into routine practice. Maintain community member engagement during dissemination and sustainment phases.

Note: EPIS=Exploration, Preparation, Implementation, and Sustainment Framework (Moullin, et al., 2019); MADI=Model for Adaptation Design and Impact (Kirk et al., 2020).